

SPARTAN COACHING

HOSPICE SALES EXCELLENCE

Visual Decision Trees & Strategic Frameworks

OBJECTION HANDLING DECISION TREE

START: You get an objection
!"
ASK: Is it REAL (substance) or REFLEX (not interested)?
% % REFLEX ("No, not now")
% % % RESPONSE: "I understand. This is important though."
% % % Provide data, ask permission for follow-up
% % % GOAL: Stay on radar (nurture, not push)
%
% % REAL (specific objection about product/fit)
% % % LISTEN: "Help me understand..."
% % % ACKNOWLEDGE: "That makes sense"
% % % RESPOND: [Use Spartan objection framework]
% % % CONFIRM: "Does that address your concern?"
% % % ADVANCE: Next step or nurture

HOSPICE REFERRAL DECISION TREE (Patient Identification)

PATIENT ASSESSMENT
!"
Does patient have terminal diagnosis?
% % NO !' Continue standard care pathway
% % YES !' Is prognosis likely <6 months?
% % UNCLEAR !' Consult physician, get clinical opinion
% % YES !' Approach physician for certification discussion
% % Physician agrees !' Proceed to family discussion
% % Physician hesitant !' Use Spartan framework (educate, not push)
% % Family ready? !' YES = Admission !' NO = Nurture plan (revisit in X days)

ACCOUNT STRATEGY MATRIX (Effort vs Opportunity)

HIGH OPPORTUNITY + LOW EFFORT = QUICK WINS (A-Priority)
New hospital opening, no established hospice partner, receptive admin
HIGH OPPORTUNITY + HIGH EFFORT = STRATEGIC FOCUS (B-Priority)
Established competitor relationship but opportunity to co-exist
LOW OPPORTUNITY + LOW EFFORT = MAINTAIN (C-Priority)
Stable account with existing partner; quarterly check-ins
LOW OPPORTUNITY + HIGH EFFORT = AVOID (D-Priority)
Unsupportive leadership, fully committed to competitor